

Adrabetadex Provider Billing Reference

Coding, claim, and reimbursement readiness leave-behind for provider billing, pharmacy, reimbursement, and revenue cycle teams

Purpose: Help provider teams prepare billing workflows before first commercial administration. This document does not replace payer-specific verification, coding review, or institutional billing policy.

[Start Here \(Provider Workflow\)](#)

- 1) Confirm claim pathway first: Hospital outpatient (UB-04/837I) vs Professional (CMS-1500/837P).
- 2) Confirm payer line of business: Medicare, Medicaid PAD, or Commercial medical benefit.
- 3) Treat code selection as candidate and payer-specific: verify C9399/J3490/J3590 usage before first claim.
- 4) Verify institution policy, chargemaster logic, and modifier requirements (TB, JW/JZ, modifier 25 when relevant).
- 5) Do not submit default coding assumptions without payer and internal coding validation.

1. Launch Billing Timeline

Period	Drug Coding / Payment State	Provider Action
Approval to Dec. 31, 2026	No permanent product-specific HCPCS expected. Interim unclassified / NOC coding likely.	Confirm payer billing instructions before administration. Prepare NDC, dose, route, units, invoice, PA approval, and medical-necessity documentation.
Jan. 1 to Mar. 31, 2027	OPPS pass-through expected for HOPD pathway, if granted. Permanent HCPCS not yet active.	Continue interim coding as directed. Confirm pass-through billing instructions, revenue code, NDC, 340B modifier, and JW/JZ wastage reporting.
April 1, 2027 forward	Permanent product-specific HCPCS expected, if assigned.	Update chargemaster, PA templates, claim edits, billing logic, medical policy references, and denial/appeal templates.

2. Drug Coding and Claim Elements

Code / Field	Code Type	Provider Use	Billing Notes
C9399	HCP CS Level II	Candidate interim hospital outpatient code for FDA-approved drugs/biologicals without assigned HCPCS.	Institutional candidate bridge code before product-specific HCPCS. Verify revenue-code/NDC/modifier requirements with payer, MAC, and hospital policy before use.

Code / Field	Code Type	Provider Use	Billing Notes
J3490	HCP CS Level II	Candidate interim NOC code for unclassified drugs.	Potential professional or payer-directed medical-benefit pathway. Verify payer preference and required claim details before billing.
J3590	HCP CS Level II	Candidate interim NOC code for unclassified biologics.	Use only if payer direction and institutional coding review support biologic NOC billing. Do not substitute for J3490 by default.
Product-specific HCPCS	HCP CS Level II	Permanent Adrabetadex drug code once assigned.	Expected April 1, 2027 . Replace C9399/J3490/J3590 when active.
NDC	Drug identifier	Package-level drug identifier.	Must match actual vial/package. Required or commonly needed for Medicaid PAD, commercial manual pricing, payer validation, and audit.
TB	HCP CS modifier	340B-acquired drug reporting.	Use only when payer-specific or Medicare policy requires 340B identification and institutional policy confirms modifier use.
JW	HCP CS modifier	Reports discarded drug amount.	Use on a separate drug line for discarded amount from a single-dose vial/package. Record administered amount and discarded amount.
JZ	HCP CS modifier	Reports no discarded drug amount.	Use when no amount was discarded from a single-dose vial/package and payer/CMS rules require no-waste attestation.

3. Administration, Sedation, and E/M Codes

A. Procedure / Administration Codes

Use only when payer/institutional coding review supports it.

Do Not Default To (Validation Required)

Do not default to 96450, J3490, J3590, C9399, TB, or modifier 25 without payer-specific and institution-specific validation. These are candidate pathways only and may vary by setting, claim form, payer policy, MAC guidance, and internal coding rules.

C o d e	C o d e T y p e	Provider Use	Billing Notes
6 2 2 7 2	CP T	Therapeutic spinal puncture / intrathecal administration.	Likely primary administration-code candidate if Adrabetadex is administered therapeutically by lumbar puncture. Document therapeutic purpose, site, route, drug, dose, administration details, tolerance, complications, and monitoring.
6 2 2 7 0	CP T	Diagnostic lumbar puncture.	Use only when diagnostic LP/CSF collection is separately medically necessary and performed. Not default for Adrabetadex administration.
6 2 3 2 8	CP T	Diagnostic LP with fluoroscopic or CT guidance.	Use only when diagnostic LP with imaging guidance is separately performed. Imaging modality and guidance must be documented.
9 6 4 5 0	CP T	Chemotherapy administration into CNS.	Do not default to this code. Use only if payer/coding review explicitly supports 96450 for the documented service.

B. Moderate Sedation Codes

C o d e	C o d e T y p e	Provider Use	Time / Billing Notes
9 9 1 5 1	CPT	Moderate sedation by same clinician performing procedure, patient <5 years, initial 15 minutes.	Minimum 10 minutes intraservice sedation time. Document start/stop time, total minutes, medication, monitoring, and observer.

C o d e	Cod e Typ e	Provider Use	Time / Billing Notes
9 9 1 5 2	CPT	Moderate sedation by same clinician performing procedure, patient ≥5 years , initial 15 minutes.	Minimum 10 minutes intraservice sedation time. Do not bill if under 10 minutes.
9 9 1 5 3	CPT add-on	Each additional 15 minutes, same clinician.	Use only after initial sedation threshold is met and additional time is documented.
9 9 1 5 5	CPT	Moderate sedation by different clinician, patient <5 years , initial 15 minutes.	Minimum 10 minutes intraservice sedation time. Use only when sedation provider differs from procedure provider.
9 9 1 5 6	CPT	Moderate sedation by different clinician, patient ≥5 years , initial 15 minutes.	Minimum 10 minutes intraservice sedation time. Use only when sedation provider differs from procedure provider.
9 9 1 5 7	CPT add-on	Each additional 15 minutes, different clinician.	Use only after initial sedation threshold is met and additional time is documented.

C. E/M Codes

E/M means Evaluation and Management. These codes bill for separate clinical assessment, decision-making, care planning, or ongoing management. E/M is not automatically billable just because the patient presents for Adrabetadex administration.

C o d e	Code Type	Provider Use	Time / Billing Notes
9 9 2 0 2	CPT	New patient office/outpatient E/M, straightforward MDM.	15-29 min if time-based.
9 9 2 0 3	CPT	New patient office/outpatient E/M, low MDM.	30-44 min if time-based.
9 9 2 0 4	CPT	New patient office/outpatient E/M, moderate MDM.	45-59 min if time-based.
9 9 2 0 5	CPT	New patient office/outpatient E/M, high MDM.	60-74 min if time-based.
9 9 2 1 1	CPT	Established patient minimal E/M, often clinical staff service.	Use only when separately supported.
9 9 2 1 2	CPT	Established patient office/outpatient E/M, straightforward MDM.	10-19 min if time-based.
9 9 2 1 3	CPT	Established patient office/outpatient E/M, low MDM.	20-29 min if time-based.

C o d e	Code Type	Provider Use	Time / Billing Notes
9 9 2 1 4	CPT	Established patient office/outpatient E/M, moderate MDM.	30-39 min if time-based.
9 9 2 1 5	CPT	Established patient office/outpatient E/M, high MDM.	40-54 min if time-based.
9 9 4 1 7	CPT add- on	Prolonged outpatient E/M under CPT/payer rules.	Use only with 99205 or 99215 when payer prolonged-time rules are met.
G 2 2 1 2	HCP CS add- on	Medicare prolonged outpatient E/M.	Medicare-specific prolonged E/M add-on. Do not use interchangeably with 99417 unless payer allows.
G 2 2 1 1	HCP CS add- on	Office/outpatient E/M visit complexity.	Use only with qualifying 99202-99205 or 99211-99215 when documentation supports longitudinal care for a serious/complex condition. Not for routine procedure-only visits.

4. Modifier Reference

Modifi er / Code	Code Type	Provider Use	Billing Notes
25	CPT modifi er	Separately identifiable E/M on same day as procedure.	Use only when E/M work is significant, separately identifiable, and supported by payer and institutional policy.

Modifier / Code	Code Type	Provider Use	Billing Notes
59	CPT modifier	Distinct procedural service.	Use only when a service is separate and not normally bundled.
XS	HCPCS modifier	Separate structure.	Use when payer requires anatomic specificity instead of 59.
XE	HCPCS modifier	Separate encounter.	Use when service occurs during a separate encounter.
XP	HCPCS modifier	Separate practitioner.	Use when service is performed by a different practitioner.
XU	HCPCS modifier	Unusual non-overlapping service.	Use when service does not overlap usual components of the primary service.
JW	HCPCS modifier	Drug discarded.	Separate drug line for discarded amount.
JZ	HCPCS modifier	No drug discarded.	Required when no discard occurred and payer/CMS rules require reporting.
TB	HCPCS modifier	340B-acquired drug.	Apply where required by Medicare or payer policy.
KX	HCPCS modifier	Requirements met.	Use only if payer policy requires KX and medical record supports criteria.
GA	HCPCS modifier	ABN issued as required.	Medicare liability notice modifier. Use only under ABN rules.
GZ	HCPCS modifier	Expected denial, no ABN obtained.	Use only under Medicare denial/ABN rules.

Modifier / Code	Code Type	Provider Use	Billing Notes
GY	HCPCS modifier	Statutorily excluded / not Medicare benefit.	Use only for statutory noncoverage or outside benefit category.
GX	HCPCS modifier	Voluntary notice of liability issued.	Use only when voluntary notice rules apply.

5. Professional RVU Planning Reference

Planning only. Verify final values against the current CMS MPFS, payer contracts, and locality.

Code	Code Type	Provider Use	Approx. Work RVU	Billing Notes
62272	CPT	Therapeutic LP / intrathecal administration	~1.54	Primary administration-code candidate.
62270	CPT	Diagnostic LP	~1.37	Diagnostic only.
62328	CPT	Diagnostic LP with fluoro/CT guidance	~1.46	Diagnostic imaging-guided LP only.
96450	CPT	CNS chemotherapy administration	~1.50	Use only if payer/coding review supports.
99151	CPT	Moderate sedation, same clinician, <5	~0.50	Initial 15 min; minimum 10 min.
99152	CPT	Moderate sedation, same clinician, ≥5	~0.25	Initial 15 min; minimum 10 min.
99153	CPT add-on	Additional sedation, same clinician	0.00	Each additional 15 min.
99155	CPT	Moderate sedation, different clinician, <5	~1.90	Initial 15 min; minimum 10 min.
99156	CPT	Moderate sedation, different clinician, ≥5	~1.65	Initial 15 min; minimum 10 min.

Cod e	Code Type	Provider Use	Approx. Work RVU	Billing Notes
99157	CPT add-on	Additional sedation, different clinician	0.00 / verify	Each additional 15 min.
G2211	HCPCS add-on	Visit complexity	Add-on	Qualifying outpatient E/M only.
G2212	HCPCS add-on	Medicare prolonged outpatient E/M	Add-on	Medicare prolonged-time threshold.
99417	CPT add-on	CPT prolonged outpatient E/M	Add-on	Payer-specific prolonged-time threshold.

6. Claim Pathway Split and Setting-Specific Checklists

Pathway	Primary Claim Form / Transaction	Use Notes
Hospital outpatient	UB-04 / 837I	Institutional drug-claim pathway; verify C9399/pass-through/revenue-code and modifier logic.
Professional	CMS-1500 / 837P	Professional service pathway; verify NOC coding candidate and professional claim rules.
Medicaid PAD	State Medicaid claim format (payer-dependent)	Follow state PAD/NDC unit rules and duplicate-discount controls.
Commercial medical benefit	Payer/plan medical claim format (payer-dependent)	Verify buy-and-bill vs specialty-pharmacy sourcing and prior authorization requirements.

Hospital Outpatient / 340B Center of Excellence

Professional (CMS-1500 / 837P)

Claim Element	Confirm
CMS-1500 / 837P professional claim pathway confirmed	☐
Payer-directed candidate NOC code (for example J3490/J3590) verified before billing	☐
NDC, dose, route, units, and narrative support included when required	☐
Prior authorization and site-of-care determinations documented	☐
Modifier 25 used only when separately identifiable E/M is supported and policy allows	☐

Payer-specific specialty-pharmacy or white-bagging instruction documented ☐

Claim Element	Confirm
UB-04 / 837I hospital outpatient claim	☐
Revenue code 0636 , if detailed drug coding applies	☐
C9399 or payer-directed NOC code before April 1, 2027	☐
Product-specific HCPCS beginning April 1, 2027	☐
NDC, dose, route, units, vial size, and date furnished	☐
OPPS pass-through billing instructions beginning Jan. 1, 2027, if granted	☐
340B modifier, such as TB, when required	☐
JW or JZ wastage modifier, when required	☐
Prior authorization approval	☐
Site-of-care approval, if required	☐
Invoice / acquisition support for manual pricing	☐
Genetic confirmation and medical necessity documentation	☐
Continuation / reauthorization evidence plan	☐

Medicaid / Medicaid Managed Care

Claim Element	Confirm
State-specific physician-administered drug requirements	☐
HCPCS/NOC code plus NDC	☐
Correct NDC unit of measure and quantity	☐
Prior authorization approval	☐
EPSDT / pediatric medical necessity documentation, if applicable	☐
340B carve-in / carve-out status	☐

Claim Element	Confirm
Duplicate-discount controls	☐
Managed Medicaid plan-specific billing rules	☐
Genetic confirmation and specialist documentation	☐
Continuation / reauthorization evidence plan	☐

Commercial Medical Benefit

Claim Element	Confirm
Prior authorization approval	☐
Buy-and-bill versus white-bagging requirement	☐
Specialty pharmacy sourcing rule	☐
Site-of-care approval	☐
Interim code or product-specific HCPCS	☐
NDC, dose, route, units, and invoice support	☐
Medical necessity letter	☐
Genetic confirmation	☐
Specialist attestation	☐
Continuation criteria / renewal requirements	☐
Appeal pathway if denied	☐

7. Documentation Minimums

Scenario	Required Documentation
Drug claim	Drug name, NDC, vial size, dose ordered, dose administered, route, units, date furnished, invoice/acquisition support, PA approval, medical necessity.

Scenario	Required Documentation
Therapeutic LP / intrathecal administration	Indication, consent, site, route, drug, dose, administration details, patient tolerance, complications, post-procedure monitoring.
Moderate sedation	Medical necessity, patient age, sedation agent, provider role, independent observer, monitoring, start/stop time, total intraservice minutes, recovery status.
Same-day E/M	Separate assessment and management beyond routine procedure work. Modifier 25 only if significant and separately identifiable.
JW wastage	Administered amount, discarded amount, vial size, reason for wastage, confirmation discarded amount was not used for another patient.
JZ no wastage	Documentation that no amount was discarded from the single-dose vial/package.
340B	340B acquisition record, covered entity status, Medicaid carve-in/carve-out logic, duplicate-discount controls, payer-required modifier.
Medicaid PAD	HCPCS/NOC code, NDC, NDC units, PA, state-specific PAD rules, EPSDT medical necessity if applicable, 340B duplicate-discount controls.
Commercial medical benefit	PA approval, site-of-care approval, buy-and-bill versus white-bagging rule, specialty pharmacy instruction, NDC, dose, invoice, genetic confirmation, specialist attestation.

8. Provider Billing Reminder

Before first commercial administration, confirm:

Question	Why It Matters
What code should be used before permanent HCPCS?	Prevents launch-period claim ambiguity.
Does the payer require buy-and-bill or white-bagging?	Determines whether the provider can bill for the drug.
Is the site 340B eligible and billing compliantly?	Changes acquisition economics and claim requirements.
Is OPPS pass-through active and applicable?	Supports HOPD Medicare payment planning.
Is NDC reporting correct?	Required for Medicaid PAD and often used for payer validation.
Is JW/JZ reporting required?	Prevents wastage-reporting denials.

Question	Why It Matters
Is E/M separately supported?	Prevents inappropriate same-day E/M billing.
Are PA and continuation criteria documented?	Reduces initial denial and reauthorization risk.

Reimbursement Disclaimer

This reference is for informational planning only and does not guarantee coverage, coding, payment, or reimbursement. Providers are responsible for selecting codes that accurately reflect the services furnished and documented in the medical record. Coverage and payment may vary by payer, plan, state Medicaid program, MAC, site of care, contract, and billing rules.